ROOT CAUSE UNLOCKED

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Unlock Your *Candida*

A Functional Medicine Guide to Identifying and Eliminating Chronic Yeast Overgrowth

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Unlock Your Candida

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> BEFORE YOU START

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KEY 1

Candida Is a Symptom, Not the Cause

Candida albicans lives in your gut right now. It lives in every healthy person's gut. The presence of Candida is not the problem. The problem is when the conditions inside your body shift enough to allow it to move from a commensal organism held in check by a balanced microbiome into an opportunistic pathogen that overgrows, penetrates gut tissue, and begins driving systemic dysfunction.

That shift does not happen randomly. Something has to change the terrain. Antibiotics that wipe out competing bacteria.[1] A sugar-heavy diet that feeds yeast preferentially. Chronic stress that suppresses secretory IgA, the mucosal antibody that holds Candida in check at the gut wall.[2] Hormonal changes that alter the vaginal or intestinal environment. Immunosuppressive medications. Mold exposure. These are the conditions that allow Candida to transition from commensal to pathogen, and they are the conditions that functional medicine identifies and addresses.

What Happens When Candida Overgrows

Candida is a dimorphic fungus, meaning it can exist in two forms. In its yeast form it is relatively contained. In its hyphal or filamentous form, it grows root-like structures called rhizoids that penetrate the intestinal epithelium, creating microscopic breaches in the gut barrier.[3] This is how Candida transitions from a gut issue to a systemic issue: once it penetrates the gut wall, Candida antigens, metabolic byproducts, and partially digested food proteins enter the bloodstream and trigger systemic immune activation.

Candida also produces gliotoxin, an immunosuppressive mycotoxin that specifically targets the immune cells responsible for controlling fungal infections. It is, in effect, a weapon that Candida uses to disable the immune defenses most capable of clearing it. This is one reason Candida overgrowth can become self-perpetuating and why simple antifungal protocols often fail without addressing the immune component directly.

Biofilm formation is the other major reason Candida is difficult to clear. Candida produces a polysaccharide-based biofilm matrix that encases the organism and protects it from both immune attack and antimicrobial agents. Standard antifungal medications and herbal protocols alike show significantly reduced efficacy against biofilm-protected Candida populations.[4] Any effective protocol must include biofilm-disrupting strategies.

The Six Drivers of Candida Overgrowth

- Antibiotic use: destroys the bacterial competition that keeps Candida at normal population levels
- High sugar and refined carbohydrate diet: provides the preferred fuel source for yeast proliferation
- Chronic stress and HPA axis dysregulation: suppresses secretory IgA and cortisol-mediated antifungal defenses

- Estrogen dominance and hormonal imbalance: estrogen promotes Candida adherence to mucosal tissue and hyphal transition[5]
- Immune suppression: from medications, mold exposure, chronic infections, or nutrient depletion
- Gut dysbiosis: loss of Lactobacillus and Bifidobacterium species removes the microbial competition that constrains Candida populations

The Symptom Picture

Candida overgrowth produces a wide and often confusing symptom picture because it affects multiple systems simultaneously. The gut is usually the primary site but the downstream effects reach the brain, the skin, the respiratory system, the reproductive system, and the musculoskeletal system. The diversity of symptoms is exactly what makes Candida so frequently missed in conventional workups, where each symptom tends to be evaluated in isolation rather than as part of a connected systemic pattern.

Digestive symptoms are usually present: bloating, gas particularly after eating carbohydrates or sweet foods, irregular bowel habits, abdominal discomfort, and an urgency or looseness that worsens after sugar intake. Brain fog is one of the most consistent neurological symptoms, driven by acetaldehyde, a toxic metabolic byproduct of Candida fermentation that crosses the blood-brain barrier and interferes with neurotransmitter function. Fatigue, mood instability, carbohydrate cravings, recurrent skin issues including tinea, oral thrush, intertrigo, and recurrent vaginal yeast infections are all common features of the systemic pattern.

Common Symptom Clusters in Candida Overgrowth

- Digestive: bloating after carbohydrates, gas, alternating constipation and diarrhea, anal itching
- Neurological: brain fog, poor concentration, memory lapses, acetaldehyde intoxication
- Mood: anxiety, irritability, depression, mood swings (driven by neurotransmitter disruption and acetaldehyde)
- Skin: tinea (ringworm), nail fungus, oral thrush, skin rashes, seborrheic dermatitis
- Hormonal: recurrent vaginal yeast infections, worsening PMS, estrogen imbalance
- Histamine: flushing after meals, headaches with fermented foods, heart palpitations, skin itching or hives, nasal congestion, anxiety after eating (driven by Candida histamine production and DAO enzyme depletion)
- Oxalates: migrating muscle and joint pain, fatigue, brain fog, and in women vulvar burning or pain (from Candida oxalate production and tissue crystal deposition)
- Energy: fatigue that worsens after eating sugar or refined carbohydrates
- Immune: food sensitivities, multiple chemical sensitivities, frequent illness, slow wound healing
- Musculoskeletal: joint pain and muscle aches driven by immune complex deposition

Why Standard Treatments Fail

Single-course antifungal medications and over-the-counter yeast treatments fail for three reasons. First, they do not penetrate biofilm. A biofilm-protected Candida population can survive antifungal concentrations that are ten to one hundred times higher than the minimum inhibitory concentration for planktonic Candida.[4] Second, they do not address the terrain conditions that allowed Candida to overgrow in the first place. The diet that fed it, the dysbiosis that allowed it, the immune suppression that failed to contain it. Third, Candida can rapidly develop resistance to single-agent antifungal approaches, particularly azoles. Without the diet, drainage, biofilm disruption, immune restoration, and gut repair addressed sequentially and comprehensively, Candida overgrowth recurs.

The functional medicine approach is fundamentally different. It addresses drainage pathways first to prevent the Herxheimer reactions that make aggressive antifungal therapy miserable, then implements biofilm-disrupting strategies, followed by targeted antifungal agents botanical and homeopathic, then rebuilds the gut environment through probiotic recolonization and barrier repair, and finally restores the immune architecture needed to keep Candida from returning. Each step sets up the next one. The sequence matters as much as the interventions themselves.

Histamine Intolerance: The Hidden Layer in Candida Overgrowth

Candida albicans produces histamine directly through its own histidine decarboxylase enzyme activity. At the same time, Candida-driven intestinal damage degrades DAO (diamine oxidase), the enzyme produced by intestinal enterocytes that breaks down dietary histamine before it is absorbed. The result is a double burden: increased internal histamine production combined with reduced clearance capacity.

Clinically this presents as a symptom cluster that is frequently mistaken for die-off: flushing and facial redness after meals, headaches that worsen with fermented foods, heart palpitations, skin itching or hives, nasal congestion, and anxiety after eating. These symptoms can occur even on the anti-Candida diet because several Candida-protocol foods are naturally high in histamine: bone broth, avocado, spinach, and any vinegar-based preparations.

The practical implication for the protocol: patients experiencing significant histamine-pattern symptoms during the active antifungal phase are not failing the protocol. They are experiencing DAO depletion driven by active overgrowth and gut barrier damage. Interventions are specific: reduce high-histamine foods during active treatment, add DAO enzyme support before meals, and prioritize gut barrier repair. Histamine symptoms typically resolve as the gut barrier heals and fungal burden reduces.

High-Histamine Foods to Reduce During Active Candida Treatment

- Animal proteins: smoked or canned fish, deli meats, any cooked protein stored more than 24 hours (histamine rises rapidly with storage)
- Vegetables: spinach, eggplant, tomatoes, avocado (also a DAO blocker)

- Fermented foods: kombucha, kimchi, sauerkraut (hold reintroduction until gut barrier repair is established)
- Bone broth: clinically valuable for gut repair but high in histamine; consume freshly made and immediately rather than stored
- Vinegar and vinegar-containing condiments: apple cider vinegar, mustard, ketchup, pickled foods
- Alcohol: all forms directly block DAO activity and trigger mast cell histamine release

Note: DAO enzyme supplements (taken with meals) can significantly reduce histamine symptoms during the active treatment phase while gut barrier repair rebuilds endogenous DAO production.

KEY 2

The Anti-Candida Diet

Diet is the most immediate lever available for altering the conditions that allow Candida to thrive. Candida preferentially metabolizes simple sugars and ferments carbohydrates, so a diet high in sugar, refined grains, alcohol, and fermented foods creates a gut environment that actively favors yeast proliferation. Conversely, removing these fuels starves the existing population, reduces the fermentation load on the gut, and begins to shift the microbial balance back toward bacterial dominance.

The anti-Candida diet is not a permanent state of dietary restriction. It is an initial therapeutic phase, typically 6 to 8 weeks for mild to moderate overgrowth and 3 to 6 months for systemic or deeply established cases, during which the yeast population is deprived of its primary fuel sources while the antifungal and gut-restoration protocol is implemented. Once Candida markers normalize on retesting and symptoms resolve, foods are methodically reintroduced in a structured sequence.

Phase 1: Remove the Fuel (Weeks 1 to 6)

Eliminate Completely

- All added sugars: table sugar, honey, maple syrup, agave, high-fructose corn syrup, and all products containing them
- Refined carbohydrates: white bread, white rice, pasta, crackers, cereals, pastries, and processed grain products
- Alcohol in all forms: alcohol feeds Candida directly and suppresses immune function
- Fruit juices and high-sugar fruits: grapes, bananas, mangoes, dried fruit, and fruit juice concentrate
- Fermented foods during the acute phase: kombucha, sauerkraut, kimchi, vinegar, and soy sauce can introduce additional yeast or stimulate existing populations in sensitive individuals (reintroduce after the acute phase)
- Moldy or mycotoxin-prone foods: peanuts, peanut butter, corn, pistachios, aged cheeses, and mushrooms in the acute phase
- Dairy: lactose is a sugar that can fuel Candida; dairy also promotes mucus in sensitive individuals that creates a favorable environment for yeast attachment
- Gluten-containing grains: wheat, barley, rye, and spelt

The Candida Diet Plate

With the fuel sources removed, the diet is built around foods that either directly inhibit Candida, support the beneficial bacteria that compete with it, or provide the nutrients needed for gut repair and immune restoration.

- Non-starchy vegetables: leafy greens, broccoli, cauliflower, asparagus, zucchini, cucumber, celery, Brussels sprouts, artichokes. These provide prebiotic fiber for beneficial bacteria and contain sulforaphane and other antimicrobial compounds.
- Clean proteins: pasture-raised poultry, grass-fed beef, wild-caught fish, and eggs. Protein does not feed Candida. Adequate protein intake is essential for maintaining secretory IgA and immune function.
- Low-glycemic carbohydrates: quinoa, millet, buckwheat, and sweet potato in moderation. These provide adequate carbohydrate without the rapid glucose spike that fuels yeast fermentation.
- Antifungal foods: garlic (allicin is a potent antifungal)[6], coconut oil (caprylic acid)[7], raw onion, ginger, turmeric, and apple cider vinegar in small amounts after the first two weeks.
- Healthy fats: avocado, olive oil, coconut oil, and omega-3-rich fatty fish. Caprylic acid in coconut oil has direct antifungal activity against Candida albicans.[7]
- Low-sugar fruits: lemons, limes, green apples, and berries in moderation. These are the most tolerated fruits during the acute phase.

Antifungal Foods Worth Highlighting

Garlic deserves special attention. Allicin, the active compound produced when garlic is crushed or chopped, has demonstrated antifungal activity against Candida albicans in multiple in-vitro studies, including against azole-resistant strains.[6] Raw garlic consumed daily, at one to two cloves per day, provides a meaningful dietary antifungal component and also has prebiotic effects that support Lactobacillus species.

Coconut oil contains caprylic acid (about 7% of its fat content), capric acid, and lauric acid, all of which have demonstrated antifungal activity against Candida.[7] Cooking with coconut oil, using it in smoothies, or taking it as a supplement at one to three tablespoons per day provides a consistent low-level antifungal dietary input that complements the protocol without the Herxheimer intensity of high-dose botanical antifungals.

Apple cider vinegar contains malic acid and acetic acid, which have antifungal properties and help maintain an acidic gut pH that is less hospitable to Candida. One to two tablespoons in water before meals, introduced after the first two weeks when the initial die-off phase is stabilizing, is a practical daily addition.

Phase 2: Reintroduction (After the Acute Phase)

Once Candida markers have normalized and symptoms have significantly improved, foods are reintroduced one at a time over several weeks. The sequence matters. Introduce one food every three to four days and monitor for symptom return: bloating, fatigue, brain fog, sugar cravings, or skin changes are all signals that the item introduced is premature.

Reintroduction order: lower-sugar fruits first, then legumes, then fermented foods (which now actively support microbial rebalancing), then whole gluten-free grains, and finally higher-sugar natural sweeteners. Gluten and dairy are best evaluated last with an extended reintroduction window because their effects on gut permeability can confound the picture. Alcohol and refined sugars do not have a meaningful reintroduction phase and are best minimized permanently.

Anti-Candida Quick Start

- Remove: all sugars, refined carbs, alcohol, gluten, dairy, peanuts, corn, aged cheeses, fermented foods (initial phase)
- Build the plate around: non-starchy vegetables, clean protein, low-glycemic carbohydrates, healthy fats
- Add daily: raw garlic, coconut oil, ginger, turmeric, lemon water
- Hydrate: filtered water, herbal teas (pau d'arco tea is directly antifungal), bone broth
- Avoid fruit juices: even natural fruit juice delivers enough sugar to feed Candida between meals
- Eat regularly: skipping meals causes blood sugar drops that intensify carbohydrate cravings. Three structured meals per day is better than grazing
- Duration: minimum 6 weeks strict adherence before evaluating for reintroduction; longer for systemic or persistent cases

KEY 3

Lab Testing for Candida

Candida overgrowth is one of the most under-tested and yet over-assumed conditions in functional medicine. Some practitioners treat every patient for Candida based on symptoms alone. Others dismiss it entirely because standard serum antibody tests are notoriously unreliable and often negative even in patients with documented overgrowth. Accurate testing using the right methodologies changes both scenarios: it identifies Candida objectively when it is present, quantifies the severity, reveals the underlying terrain conditions, and provides the objective baseline needed to document treatment response.

Each test below is linked directly to our patient ordering portal. Click the test name to order.

Test	What It Measures	Why It Matters for Candida
Gut Zoomer 3.0 (Vibrant)	300+ microbiome species including Candida albicans and other Candida species; secretory IgA; calprotectin; eosinophil protein X; lactoferrin; zonulin; permeability markers	Directly quantifies Candida species in stool by DNA sequencing. Also reveals the dysbiosis pattern (depleted Lactobacillus and Bifidobacterium) that allowed Candida to overgrow. Gut barrier integrity markers confirm whether systemic translocation is occurring.
Organic Acids Test (Vibrant)	Candida metabolites: D-arabinitol, arabinose, and other fungal organic acids; bacterial dysbiosis markers; mitochondrial function; neurotransmitter metabolites; oxalate burden	The OAT is the gold standard functional marker for systemic Candida burden. Elevated D-arabinitol is the most specific marker for intestinal Candida overgrowth and correlates strongly with systemic candidiasis.[8] It captures what DNA stool testing can miss if Candida is in the upper gut.
GI-MAP (Diagnostic Solutions)	Quantitative PCR for Candida albicans and other fungal species; H. pylori; bacterial pathogens; parasites; zonulin; calprotectin	PCR-based testing is highly sensitive for Candida DNA in stool. The quantitative result tells you how much Candida is present, not just whether it is detected. Combined with the OAT, this gives both functional load and stool population data.
Micronutrient Panel (Vibrant)	Intracellular vitamin C, vitamin D, zinc, magnesium, B vitamins, selenium, antioxidants, and essential fatty acids	Candida overgrowth depletes zinc, magnesium, vitamin C, and B vitamins directly. These deficiencies then perpetuate overgrowth by impairing the immune defenses needed to contain Candida. Supplementing without knowing what is depleted is guesswork.
Food Zoomer (Vibrant)	IgG and IgA antibodies to 200+ foods; identifies delayed food sensitivities. Covers food, additives, and the following zoomers: corn, dairy, egg, peanut, soy, and wheat.	Candida overgrowth generates intestinal permeability that drives food sensitivity development. Food sensitivities then maintain immune activation and inflammation that perpetuates the dysbiotic environment. Identifying and removing reactive foods is an essential component of treatment.
Wheat Zoomer (Vibrant)	Wheat antibodies; zonulin; intestinal permeability antibodies; celiac genetics	Gluten drives zonulin-mediated gut permeability that is synergistic with Candida-driven permeability.[9] Testing confirms whether gluten elimination is a priority in the protocol and quantifies the barrier damage already present.
Vibrant Mycotoxin Profile	Mycotoxins including gliotoxin, ochratoxin A, and other fungal toxins; heavy metals; environmental chemicals	Candida albicans produces gliotoxin as a virulence factor. Elevated urinary mycotoxins confirm that Candida is actively producing immunosuppressive compounds. Mold co-exposure amplifies the total

Test	What It Measures	Why It Matters for Candida
		mycotoxin burden and must be distinguished from Candida-derived sources.
Candida + IBS Profile (Vibrant)	IgG and IgA antibodies to Candida albicans and other fungal species; IBS-related serum markers	Blood-based serology panel specifically designed to detect immune reactivity to Candida. Particularly valuable when stool Candida DNA is negative but OAT fungal markers are elevated, confirming systemic immune activation even when gut colonization is not quantifiable by PCR. Vibrant; $300.

All Vibrant Wellness panels listed above can be ordered directly through the Root Health patient portal at seaywellness.wellproz.com. The GI-MAP (Diagnostic Solutions) is ordered through our office. Contact us at rootcauseunlocked.com to request it.

Understanding What the Tests Show

The OAT and D-Arabinitol: The Most Important Marker

D-arabinitol is a metabolic byproduct of Candida fermentation that is excreted in urine. Unlike stool testing, which only captures what is in the colon at the time of collection, urinary D-arabinitol reflects systemic Candida burden across the entire digestive tract, including the small intestine where Candida often establishes its most disruptive overgrowth.[8] A patient can have a negative stool Candida result and highly elevated urinary D-arabinitol, which means the primary overgrowth is in the upper gut, above where stool testing detects it. This is exactly the population that gets dismissed by conventional testing and remains untreated.

The ratio of D-arabinitol to creatinine is the most clinically meaningful calculation. Elevated absolute D-arabinitol with a high ratio confirms active Candida fermentation. The OAT also identifies oxalate burden, which is directly relevant to Candida: Candida species produce oxalates as a metabolic byproduct, meaning elevated urinary oxalates on the OAT are a fungal burden marker, not just a dietary or kidney issue. High oxalate loads from Candida produce a specific symptom cluster: diffuse migrating muscle and joint pain, fatigue that does not respond to sleep, brain fog, and in women, vulvar burning or pain that is frequently misdiagnosed as recurrent yeast infection when it is actually oxalate crystal deposition in vulvar tissue. Dietary reduction of high-oxalate foods (spinach, almonds, cashews, peanuts, beets, Swiss chard, cocoa) during active treatment reduces the load while the fungal source is cleared. Calcium citrate taken with meals (not away from meals) binds dietary oxalates in the gut before absorption. Vitamin B6 as P5P reduces endogenous oxalate production. Always reduce dietary oxalates gradually rather than abruptly: rapid reduction triggers a mobilization reaction as stored tissue crystals release and can temporarily worsen pain and fatigue.

Gut Zoomer: Seeing the Full Microbial Picture

Beyond identifying Candida species directly, the Gut Zoomer reveals the microbiome context that explains why Candida was able to overgrow. Low Lactobacillus species, depleted Bifidobacterium, and high pathobiont populations create the bacterial vacuum that Candida fills. Low secretory IgA confirms that the mucosal immune defense is depleted. Elevated calprotectin or lactoferrin indicates active gut inflammation. Elevated zonulin confirms that Candida has contributed to intestinal permeability. Together these markers build the complete terrain picture and make it possible to design a protocol that addresses all the drivers, not just the Candida itself.

Gliotoxin and the Mycotoxin Profile: Confirming Virulence and Mold Co-Exposure

Gliotoxin is the mycotoxin that Candida produces specifically to suppress the neutrophils and macrophages responsible for clearing fungal infections. Finding elevated gliotoxin on the Vibrant Mycotoxin Profile confirms that the Candida present is actively expressing its virulence factors, which is clinically significant because it means immune suppression is part of the mechanism. Antifungal therapy without concurrent immune support is markedly less effective in these patients. The panel also measures environmental mycotoxins from mold exposure (ochratoxin A, aflatoxin, trichothecenes) and distinguishes these from Candida-derived sources. A patient with only Candida-derived gliotoxin has a different clinical picture from one who also carries ochratoxin from building mold. That distinction determines whether mold remediation is a required component of resolution. Clinically the protocol diverges at this point: a patient with Candida-only gliotoxin (elevated gliotoxin on Vibrant Mycotoxin Profile, no ochratoxin or aflatoxin) is treated with the standard Candida protocol: drainage, biofilm disruption, targeted antifungals, immune restoration. The gliotoxin normalizes as Candida burden reduces. A patient with gliotoxin alongside ochratoxin, aflatoxin, or trichothecenes from environmental mold exposure requires parallel mold remediation: binders specific to mycotoxins (MycoPul, Ultra Binder), environmental assessment of their living or work space, and a drainage protocol capable of handling the combined toxin burden. Treating Candida aggressively in a patient with concurrent active mold exposure without addressing the mold source produces incomplete and often frustrating results, because the immune suppression from ongoing environmental mycotoxins prevents the immune restoration that keeps Candida from returning.

GI-MAP: Quantitative Fungal Load and What Else Is Present

The GI-MAP uses quantitative PCR to detect Candida DNA in stool with high sensitivity. The quantitative result matters: there is a meaningful difference between a trace signal (possibly commensal presence in a patient with adequate terrain) and a high-copy result confirming active overgrowth. More importantly, the GI-MAP reveals what is co-existing alongside the Candida. H. pylori, bacterial pathogens, and parasites frequently co-exist with Candida and must be addressed in sequence. Treating Candida while a significant H. pylori infection or parasite load is present produces incomplete results because those organisms perpetuate the same terrain conditions that allow Candida to thrive. Running the GI-MAP alongside the OAT provides both functional metabolic load and stool population data simultaneously: the most complete single-panel view of what is happening in the gut.

Micronutrient Panel: What Candida Has Taken From You

Candida is metabolically expensive for the host. As it ferments sugars and produces acetaldehyde, it depletes the nutrients required to clear those toxins and mount an immune response. Zinc is consumed in antifungal immune activity and is almost universally low in chronic Candida patients. Magnesium is depleted by the stress response Candida perpetuates. Vitamin C is oxidized managing the free radical load from acetaldehyde. B vitamins, particularly B6, B12, and folate, are consumed in methylation and detoxification pathways working overtime. Selenium, essential for glutathione synthesis, is frequently below functional range.

These deficiencies are not incidental: they are part of the mechanism that keeps Candida entrenched. An immune system depleted of zinc and selenium cannot clear Candida effectively regardless of what antifungal agents are used. The Micronutrient Panel identifies exactly which depletions are present so supplementation can be targeted rather than generic. It also establishes a baseline for measuring recovery: replenishment of depleted nutrients is one of the objective markers that the protocol is working.

Food Zoomer: The Candida-Sensitivity Connection

One of the most consistent downstream consequences of Candida overgrowth is the development of delayed food sensitivities. As Candida hyphae penetrate the intestinal epithelium, they create microscopic gaps in the gut barrier that allow partially digested food proteins into the bloodstream. The immune system mounts IgG and IgA antibody responses to these proteins, producing a growing list of foods that generate inflammation hours or days after consumption, which is why patients often cannot connect their symptoms to specific foods.

The Food Zoomer identifies IgG and IgA reactivity to over 180 foods, allowing the most reactive foods to be removed during the treatment protocol. Reactive foods maintain gut inflammation that perpetuates the dysbiotic environment Candida thrives in, and they consume immune capacity that would otherwise support antifungal clearance. Food sensitivity testing in a Candida patient is not a separate workup: it is an integral part of identifying why the terrain remains inflamed and what dietary changes are required for resolution to hold.

Wheat Zoomer: Quantifying the Barrier Damage

Gluten and Candida both drive intestinal permeability through zonulin, the protein that regulates tight junctions between gut epithelial cells.[9] When both are present simultaneously (which is common), the permeability damage is additive. The Wheat Zoomer measures antibodies to wheat proteins and gliadin peptides, zonulin levels, and intestinal permeability antibodies including anti-actin and anti-lipopolysaccharide. Together these markers quantify how much barrier damage is present and whether gluten is actively contributing to it.

In patients with both Candida-driven and gluten-driven permeability, strict gluten elimination is structural, not optional. Every gluten exposure in a sensitized patient reopens the tight junctions the protocol is working to close. The Wheat Zoomer also screens for celiac genetics, which matters because celiac disease produces gut barrier disruption and dysbiosis that directly predisposes to Candida overgrowth. Identifying celiac or non-celiac gluten sensitivity changes the dietary recommendation from a temporary protocol measure to a permanent requirement for that patient.

Candida + IBS Profile: When Stool Testing Comes Back Negative

The Candida + IBS Profile measures serum IgG, IgA, and IgM antibodies to Candida albicans and related fungal species, answering a specific clinical question: has the immune system mounted a systemic antibody response to Candida? Stool-based testing can return negative when the primary overgrowth is in the small intestine rather than the colon, or when Candida burden has diminished enough that stool DNA is below detection thresholds while systemic immune activation persists.

A patient with elevated OAT D-arabinitol, negative stool Candida, and elevated serum IgG or IgA on this panel has a complete diagnostic picture: systemic Candida fermentation is occurring and the immune system has responded, even though stool testing alone would have appeared to clear them. This is the panel that catches the cases most likely to be dismissed in conventional workups. It also differentiates systemic Candida immune burden from IBS as a primary diagnosis, a distinction that many patients carrying an IBS label for years have never had made, and one that changes the treatment approach entirely.

KEY 4

Supplementation Protocols

The supplementation approach to Candida overgrowth follows a deliberate sequence: drainage and preparation, biofilm disruption, targeted botanical antifungal therapy, gut barrier repair, and probiotic recolonization. Skipping the drainage phase produces Herxheimer reactions that are often intense enough to cause patients to abandon treatment. Skipping biofilm disruption means the antifungal protocol will only address the free-floating Candida population, leaving the protected biofilm community intact to repopulate afterward. Sequence is as important as product selection.

Everything below is available as a single-ingredient or standardized botanical product. Nothing here is practitioner-exclusive. The sequence matters far more than the sourcing.

Phase 1: Drainage and Preparation (Weeks 1 to 4)

Drainage must be established before antifungal agents are introduced. As Candida dies off it releases cell wall fragments, acetaldehyde, and endotoxins that must be cleared through the lymphatic, hepatic, and bowel pathways. Without adequate clearance capacity, this load produces severe Herxheimer symptoms and often halts the protocol.

Support	What to look for	What it does	Typical range
Digestive enzymes	Full-spectrum plant enzyme blend (amylase, protease, lipase, cellulase). A protease-free version with slippery elm, marshmallow root, chamomile, and ginger suits an inflamed or sensitive gut better	Reduces the fermentation substrate available to Candida by making macronutrient breakdown more complete	1 to 2 capsules with meals, up to 3 with meals in chronic cases
Proteolytic enzymes, away from food	Serrapeptase, nattokinase, or bromelain on an empty stomach	At higher doses these degrade the protein scaffold inside biofilm, an essential preparatory step	Per label, on an empty stomach, three times daily
N-acetyl cysteine	Standard NAC	Breaks the disulfide bonds within the Candida biofilm matrix, increasing antifungal penetration, and serves as the glutathione precursor for acetaldehyde clearance. Dual function	600 to 900 mg two to three times daily, away from meals
Glutathione	Liposomal or acetylated glutathione	The primary intracellular antioxidant for clearing acetaldehyde and Candida-derived metabolites through the liver. Most relevant during die-off, when hepatic demand is highest	Per label, twice daily
Antioxidant support	A polyphenol blend such as grape seed extract, green tea catechins, milk thistle, quercetin, and turmeric	Broad antioxidant support for the reactive oxygen species generated during die-off. Pairs with glutathione without redundancy	Per label, twice daily
Electrolytes and hydration	A mineral electrolyte concentrate without added sugar	Supports the fluid environment that lymphatic and renal clearance depends on	Per label, through the day

Phase 2: Targeted Antifungal Therapy (Weeks 3 to 12)

Antifungal agents are introduced progressively, starting low and building over one to two weeks so the clearance pathways can handle the die-off load.

Support	What to look for	What it does	Typical range
Caprylic acid	Calcium caprylate or coconut-derived caprylic acid, not synthetic	A medium-chain fatty acid that disrupts Candida cell membranes.[7] A well-tolerated place to start	1,000 to 2,000 mg three times daily with meals
Oregano oil	Standardized to at least 80% carvacrol, enteric-coated for intestinal delivery	Carvacrol and thymol have antifungal activity including against azole-resistant strains, and carvacrol penetrates biofilm.[7] Enteric coating gets it past the stomach	1 to 3 capsules three times daily with meals
Berberine	Berberine HCl, not berberine sulfate	Antifungal, antibacterial, and anti-biofilm activity,[10] plus effects on the dysbiotic bacteria that travel with Candida overgrowth	500 mg three times daily with meals
Undecylenic acid	Castor-oil derived	A fatty acid antifungal that is particularly active in the small intestine	Per label, with meals
Pau d'arco	Tabebuia impetiginosa bark extract	Contains lapachol and beta-lapachone, the basis of its traditional antifungal use. Often combined with jatoba, usnea, or spilanthes in multi-herb formulas	Per label, twice daily
Beta-glucan mushroom blend	Cordyceps, maitake, reishi, shiitake, turkey tail	Beta-glucans support natural killer cell activity and T-cell function, which matters because Candida suppresses immune signaling through gliotoxin	Per label, twice daily
Zinc	Zinc glycinate, picolinate, or orotate	Direct antifungal activity, and the mineral most commonly depleted in this population. Required for immune function and barrier integrity	25 to 30 mg elemental zinc daily with food
Vitamin D3 with K2	Cholecalciferol with MK-7	Immune modulation. Test your level, dose to the result with your clinician, and retest rather than guessing	Dose to your measured level

Support	What to look for	What it does	Typical range
Binder	Activated charcoal, bentonite clay, or a multi-component binder with zeolite, modified citrus pectin, and larch arabinogalactan	Binds cell wall fragments, acetaldehyde, and gliotoxin released during die-off before they can be reabsorbed. The single most useful item for keeping die-off tolerable	Per label, 30 minutes before or 1 hour after all food, supplements, and medication

Phase 3: Gut Repair and Recolonization (Months 2 to 4)

Support	What to look for	What it does	Typical range
Saccharomyces boulardii	CNCM I-745 or DBVPG 6763 strain	A beneficial yeast that competes with Candida albicans, reduces its adhesion to the intestinal lining, and supports secretory IgA.[11] Safe alongside antifungal therapy	10 billion CFU twice daily
Multi-strain probiotic	Lactobacillus and Bifidobacterium strains with clinical evidence behind them	Recolonizes the bacterial species that provide the competitive exclusion keeping Candida at healthy population levels	25 to 50 billion CFU daily, at least 2 hours away from antifungals
Colostrum	Bovine colostrum standardized for immunoglobulin content	Supports the gut-associated lymphoid tissue and the secretory IgA capacity breached by Candida hyphal invasion	Per label, morning and evening
L-glutamine	Pharmaceutical-grade powder	The primary fuel for intestinal enterocytes and the main nutrient for repairing damaged gut epithelium. Glycine and proline support the structural repair alongside it	5 to 10 g twice daily in water, away from meals

Critical Note on Die-Off Management

Herxheimer reactions from Candida die-off are real and can be significant: fatigue, headache, brain fog, nausea, skin breakouts, and mood disturbances typically peaking in the first two to three weeks of antifungal therapy as Candida populations rapidly die and release their cellular contents.

Management strategies: reduce the antifungal dose and build up slowly; make sure a binder is already in place; maintain hydration; support the liver with NAC and glutathione; and reduce die-off intensity by starting with diet changes alone for two weeks before adding botanical antifungals.

The order is the point. Establish the enzymes, NAC, glutathione, and a binder first, because those are what create the capacity to handle die-off. Do not introduce botanical antifungals before that groundwork is in place.

KEY 5

Therapeutic Modalities

The supplement and diet protocol addresses the internal biochemistry of Candida overgrowth. These adjunctive modalities address the biofilm, the immune regulation, the drainage pathways, and the environmental factors that directly influence whether the protocol succeeds and whether Candida returns.

Biofilm Disruption Strategies

Candida biofilm is the primary reason antifungal protocols fail. The biofilm is a polysaccharide matrix, primarily composed of mannan, glucan, and chitin, that Candida constructs around colonies as a protective structure.[4] Disrupting or degrading this matrix before or alongside antifungal therapy dramatically increases the exposure of the underlying Candida population to antifungal agents. Several strategies are clinically supported.

- Proteolytic enzymes away from food: serrapeptase, nattokinase, and bromelain taken on an empty stomach have biofilm-degrading activity by breaking down the protein scaffold within the matrix. NAC breaks the disulfide bonds that hold the biofilm together. These are best used during Phase 1 before antifungal introduction.
- Lactoferrin: an iron-binding glycoprotein that disrupts biofilm formation and has direct antifungal effects. Most effective when paired with antifungal agents.
- Oregano oil: beyond its antifungal activity, carvacrol has demonstrated biofilm-penetrating properties that make it particularly useful in combination protocols[7]

Infrared Sauna

Regular infrared sauna use accelerates Candida clearance through several mechanisms. Heat directly stresses fungal organisms: Candida albicans is significantly more sensitive to temperature elevation than human cells, and mild hyperthermia impairs fungal enzyme function and reduces Candida virulence factor expression. Infrared sauna also promotes sweating as a route of elimination for acetaldehyde and other Candida-derived toxins that would otherwise require hepatic processing. This effectively reduces the die-off burden on the liver.

Additionally, infrared sauna activates the parasympathetic nervous system and reduces cortisol, which matters because elevated cortisol directly suppresses the antifungal immune responses that the protocol is trying to restore. Vagal activation through sauna also improves gut motility, which helps clear Candida debris and maintain bowel regularity essential for toxin elimination.

Starting protocol: 15-20 minutes at 120-130 degrees Fahrenheit two to three times per week. Increase to 30-40 minutes and three to five times per week as tolerance builds. Always take binders two to four hours before sauna sessions during the active antifungal phase to pre-load the elimination capacity for the toxins that sweating will mobilize. Hydrate with electrolyte-containing water before and after.

Binders and Detoxification Support

Binders are non-absorbable substances that bind toxins in the gut lumen and prevent their reabsorption. They are particularly important during Candida die-off because the gut lumen fills rapidly with Candida cellular debris, acetaldehyde, and gliotoxin as the antifungal protocol takes effect. Without adequate binder capacity, these toxins undergo enterohepatic recirculation and return to the bloodstream repeatedly, producing prolonged and severe Herxheimer symptoms.

- Activated charcoal: broad-spectrum binder that is most effective for acetaldehyde and bacterial toxins; take 30 minutes to one hour away from all supplements and medications
- Bentonite clay: binds mycotoxins and heavy metals with reasonable affinity; 1 teaspoon in water on an empty stomach
- Cholestyramine or modified citrus pectin: stronger prescription-level binders reserved for patients with significant mycotoxin burden confirmed on Vibrant Mycotoxin Profile testing
- Multi-component binders combining zeolite, bentonite clay, modified citrus pectin, and activated charcoal, taken well away from food and other supplements
- Charcoal or binder timing: always two hours away from all therapeutic supplements and at least one hour before meals; typically taken mid-morning and mid-afternoon

Castor Oil Packs

Castor oil packs applied to the abdomen over the liver and gut area stimulate lymphatic circulation, increase local blood flow, reduce bowel transit time, and have a clinically observed anti-inflammatory effect on the underlying tissues. They are one of the most underused and accessible tools for supporting the drainage phase of Candida treatment.

Application: saturate a flannel cloth with cold-pressed castor oil, apply over the right upper quadrant of the abdomen covering the liver, cover with plastic wrap to prevent staining, apply a heating pad set to medium, and rest for 45 to 60 minutes. Three to four times per week during the drainage and antifungal phases. Wash the area after removal. This is a simple, inexpensive, and well-tolerated intervention that noticeably reduces the severity of die-off symptoms when used consistently.

Stress Management and HPA Axis Support

Cortisol promotes Candida virulence. Studies have demonstrated that cortisol directly stimulates the morphological transition of Candida from its yeast form to the more invasive hyphal form, the same transition that allows it to penetrate the gut wall. Chronically elevated cortisol from stress, sleep deprivation, or HPA axis dysregulation is therefore not just a cofactor in Candida overgrowth but a direct driver of its pathogenicity.

Conversely, chronic low-cortisol states from adrenal exhaustion suppress the antifungal immune responses that neutrophils and macrophages depend on. Both extremes impair the body's ability to contain Candida. HPA axis normalization is therefore a therapeutic target in Candida protocols, not an optional wellness consideration. Adrenal glandular support may be used in the hypo-adrenal pattern; adaptogenic herbs including ashwagandha, rhodiola, and licorice root address the HPA axis rhythm.

- Diaphragmatic breathing: 10 minutes twice daily, 5-count inhale, 7-count exhale, activates vagal tone and reduces cortisol
- Sleep: 7-9 hours in a dark, cool room; the lowest cortisol point occurs between midnight and 4am and is essential for immune restoration
- Moderate exercise: reduces stress hormones and improves gut motility; avoid excessive high-intensity training during active die-off phases when adrenal demand is already elevated
- Limit caffeine: caffeine elevates cortisol; reduce to one cup of coffee or green tea in the morning during the active treatment phase

Environmental Mold Assessment

Mold and Candida share biochemical territory: both produce mycotoxins, both respond to similar antifungal interventions, and environmental mold exposure directly impairs the immune defenses that control Candida. A patient in a water-damaged building who is being treated for Candida will have a dramatically reduced treatment response because the ongoing mold exposure continuously suppresses the immune system and adds to the total mycotoxin burden. Environmental assessment is not optional when the Vibrant Mycotoxin Profile shows elevated mycotoxins alongside a Candida picture.

Practical steps: have the home inspected by a certified industrial hygienist or use an ERMI (Environmental Relative Moldiness Index) test if inspection is not immediately accessible. Focus on the bedroom, the HVAC system, and any areas with prior water damage. A HEPA air purifier with activated carbon in the bedroom reduces inhalation exposure during the eight or more hours spent sleeping. Reduce indoor humidity below 50 percent. Inspect and remediate if Stachybotrys, Aspergillus, or Penicillium species are found at elevated levels.

KEY 6

Working with Root Health

Candida overgrowth is one of the conditions most prone to incomplete treatment. The diet phase helps and symptoms improve. Patients assume they have solved the problem, stop the protocol before the gut repair and recolonization phase is complete, and Candida returns within weeks or months as the underlying terrain that allowed it has not been fully rehabilitated. Working with a practitioner who can track objective markers, adjust the protocol based on what retesting shows, and ensure each phase is completed before progressing to the next is what separates successful long-term resolution from the cycle of temporary improvement and relapse.

The M4 Method Applied to Candida

At Root Health, every patient case is approached through the M4 framework, which integrates chiropractic, structural, and functional medicine into a unified care model.

- Movement: Chiropractic care addresses the nervous system regulation that governs gut motility, immune function, and stress response, all of which directly influence Candida containment. Structural corrections to the thoracic and lumbar spine improve vagal tone and reduce sympathetic drive on the gut.
- Manipulation: Chiropractic adjustments reduce sympathetic nervous system dominance that elevates cortisol and promotes Candida hyphal transition. Regular adjustments during the protocol support the HPA axis normalization that is essential for long-term Candida containment.
- Medicinal Nutrition: A supplement protocol sequenced across the drainage, antifungal, and restoration phases described in Key 4. Selection and sequencing are based on your laboratory findings and clinical assessment rather than a generic template, and every product used is a single ingredient or standardized botanical you can obtain yourself.
- Metabolic Connection: The upstream metabolic factors (blood sugar regulation, thyroid and adrenal function, hormonal balance, mitochondrial energy production) are addressed because each of these systems directly influences the terrain conditions that allow Candida to overgrow or be kept in check.

The Assessment Process at Root Health

For Candida patients, the assessment process begins with comprehensive laboratory testing to objectively confirm overgrowth and quantify the terrain conditions driving it. We use the OAT and Gut Zoomer as the primary diagnostic tools, supplemented by the GI-MAP where indicated, Micronutrient Panel, Food Zoomer, and Vibrant Mycotoxin Profile ($606) if mycotoxin burden is suspected. This data-driven approach replaces assumptions with measurements.

We also use FM Logic questionnaire analysis, ZYTO bioenergetic scanning, and QRA muscle testing as functional assessment tools that help identify areas of energetic priority and map the causal chain from the presenting condition back to its root. These bioenergetic tools complement the objective lab data and help identify which systems are functionally compromised even before lab abnormalities fully develop. Combined, these assessment methods ensure that nothing gets missed and that the protocol is customized to what your body is actually showing, not a generic Candida template.

Phases of Care

Phase 1: Drainage and Preparation (Weeks 1 to 4)

No antifungal therapy begins until the drainage pathways are open. This phase establishes lymphatic, hepatic, and bowel clearance capacity: digestive and proteolytic enzymes, NAC, glutathione support, electrolytes, and a binder. The anti-Candida diet begins on day one. Digestive enzyme support is introduced. Patients are coached on die-off management before antifungal agents are added.

Phase 2: Targeted Antifungal Therapy (Weeks 3 to 12)

Botanical antifungals are introduced progressively based on tolerance: caprylic acid or enteric-coated oregano oil first, then berberine for its combined antifungal and dysbiosis-clearing properties, with pau d'arco added where indicated. Biofilm disruption with NAC and enzyme therapy is ongoing. Die-off monitoring is active and the protocol is adjusted based on response. Binders continue. Immune support with a beta-glucan mushroom blend, zinc, and vitamin D runs throughout this phase.

Phase 3: Gut Repair and Recolonization (Months 2 to 4)

As Candida markers normalize, the focus shifts to rebuilding. Saccharomyces boulardii and a multi-strain probiotic are emphasized. Colostrum and L-glutamine rebuild the gut barrier. Antifungal intensity is maintained at a lower level while recolonization occurs. Retesting of OAT and Gut Zoomer at the 3 to 4 month mark provides objective confirmation of progress.

Phase 4: Maintenance and Prevention

Long-term Candida control depends on maintaining the microbial ecosystem that provides competitive exclusion. Ongoing probiotic therapy at maintenance dosing, continued dietary discretion around sugar and refined carbohydrates, stress management, and periodic low-dose antifungal botanical cycles if indicated by retesting. Most patients at this phase have enough awareness of their personal triggers to manage proactively and avoid the recurrence cycle that defined their experience before treatment.

The Root Discovery Session

The entry point for working with Root Health is the Root Discovery Session: a 90-minute comprehensive consultation covering your complete health history, laboratory review, assessment findings, and an individualized protocol direction.

The session cost credits in full toward any program you choose to begin.

For Candida patients specifically, we discuss which laboratory tests to order first, explain the drainage-first sequencing, and set realistic expectations for the timeline to resolution based on the severity and chronicity of your case.

Schedule at rootcauseunlocked.com or call our Garner, NC office directly.

Your 90-Day Candida Action Plan

The steps below are sequenced deliberately. Drainage before antifungals. Biofilm disruption before high-dose botanical therapy. Gut repair concurrent with and following antifungal phases. Recolonization during and after repair. Jumping ahead produces miserable Herxheimer reactions and incomplete outcomes. Follow the sequence.

Weeks 1 to 4: Diet and Drainage

Weeks 1-4 Priorities

- Begin the anti-Candida diet completely: remove all sugar, refined carbs, alcohol, gluten, dairy, peanuts, fermented foods, and high-glycemic fruits
- Start cooking daily with coconut oil and raw garlic; add ginger and turmeric to meals
- Begin NAC 600mg twice daily away from meals for biofilm preparation and glutathione support
- Start L-glutamine 5g twice daily in water on an empty stomach to begin gut barrier repair
- Add a quality probiotic (Saccharomyces boulardii first, then add multi-strain Lactobacillus/Bifidobacterium in week 2)
- Establish binder protocol: activated charcoal or bentonite clay away from supplements and meals
- Begin castor oil packs over the liver 3-4 times per week
- Begin drainage support: digestive enzymes with meals, proteolytic enzymes away from food, and electrolytes through the day
- Order laboratory tests: OAT and Gut Zoomer as first priority; add GI-MAP and Micronutrient Panel if budget allows
- Establish sleep schedule: 7-9 hours, lights out by 10:30pm

Weeks 5 to 8: Antifungal Introduction

Weeks 5-8 Priorities

- Introduce antifungal agents progressively: begin at half dose for one week then increase to full dose
- Caprylic acid or enteric-coated oregano oil: start with one dose per day for 5 days, then increase to 3x daily
- Berberine HCl 500mg with meals: add in week 5
- Add pau d’arco alongside the first antifungal in week 6 if tolerance has been good

- Oregano oil at 1 capsule daily with meals in week 6, increasing to twice daily if tolerated in week 7
- Review OAT and Gut Zoomer results with practitioner and adjust protocol to findings
- Eliminate any foods identified on Food Zoomer as reactive
- Maintain binders and drainage support throughout
- Begin infrared sauna 2-3 times per week; take binders 2-4 hours before each session
- Monitor die-off intensity: if Herxheimer symptoms are severe, reduce antifungal dose rather than pushing through

Weeks 9 to 12: Gut Repair and Immune Restoration

Weeks 9-12 Priorities

- Maintain antifungal protocol while adding gut repair emphasis
- Add colostrum for mucosal immune barrier restoration
- Increase Saccharomyces boulardii to full dosing and add a multi-strain probiotic
- Add L-glutamine if not already started
- Add a beta-glucan mushroom blend for immune restoration
- Add zinc for repletion and additional antifungal support
- Increase infrared sauna to 4 times per week if tolerated
- Begin careful reintroduction of previously eliminated foods if symptoms have improved significantly
- Schedule OAT and Gut Zoomer retest at month 4 to document objective progress
- Reduce binder use to as-needed basis if die-off symptoms have resolved
- Reassess symptom burden: most patients report 50-70% improvement by week 10-12

Tracking Your Progress

Keep a daily symptom log tracking: bloating, brain fog, energy level, sugar cravings, skin status, bowel regularity, and mood on a 1 to 10 scale. This creates a visible trajectory that helps you recognize that progress is happening even during weeks when die-off symptoms make it feel otherwise. The symptom curve in Candida treatment is rarely linear. Some patients feel worse before they feel better as die-off peaks in weeks 3 to 5. Others improve steadily from week one. Both are normal. The symptom log helps distinguish die-off worsening from protocol intolerance, which is the key clinical distinction in those first difficult weeks.

The objective retest at 3 to 4 months is the most important milestone. A normalized D-arabinitol on the OAT, reduced or absent Candida DNA on the Gut Zoomer, improved secretory IgA, and reduced intestinal inflammation markers are the objective confirmations that the protocol is working. Symptoms improve subjectively before the labs fully normalize, so the retest catches patients who feel better but have not yet resolved the underlying overgrowth and guides the transition to a maintenance rather than active treatment approach.

APPENDIX

Quick Reference

Protocol Summary: Candida

Support	Purpose	Typical range
Electrolytes	Hydration and clearance support	Per label, through the day
Digestive enzymes	Reduce fermentation substrate	1 to 2 caps with meals
Proteolytic enzymes, away from food	Biofilm preparation	Per label, empty stomach, 3x daily
N-acetyl cysteine	Biofilm disruption and glutathione precursor	600 to 900 mg 2 to 3x daily, away from meals
Glutathione	Acetaldehyde and metabolite clearance during die-off	Per label, twice daily
Polyphenol antioxidant blend	Antioxidant support during die-off	Per label, twice daily
Binder (charcoal, clay, zeolite, modified citrus pectin)	Mops up die-off debris and biofilm fragments	Per label, always away from other supplements
Caprylic acid	Membrane-disrupting antifungal	1,000 to 2,000 mg 3x daily with meals
Oregano oil, enteric-coated	Broad-spectrum botanical antifungal	1 to 3 caps 3x daily with meals
Pau d’arco	Botanical antifungal	Per label, twice daily
Berberine HCl	Antifungal, antibiofilm, antimicrobial	500 mg 3x daily with meals
Undecylenic acid	Small-intestinal antifungal	Per label, with meals
Beta-glucan mushroom blend	Immune restoration against gliotoxin suppression	Per label, twice daily
Vitamin D3 with K2	Immune modulation	Dose to your measured level
Zinc	Repletion plus antifungal activity	25 to 30 mg elemental daily with food
Saccharomyces boulardii	Direct Candida competitor	10 billion CFU twice daily

Support	Purpose	Typical range
Multi-strain probiotic	Recolonization and competitive exclusion	25 to 50 billion CFU daily
Colostrum	Mucosal barrier and secretory IgA	Per label, morning and evening
L-glutamine	Gut barrier repair	5 to 10 g twice daily, away from meals

> ***Where to get these***
>
> *Every item above is a single ingredient or a standardized botanical, available from any reputable retailer. Look for independent third-party testing, and match the form named in the table rather than the name on the front of the bottle.*
>
> *If you would rather not evaluate that yourself, Root Health keeps a curated dispensary of professional-grade products, screened for those standards and organized by the phases in Key 4.*
>
> ***Root Health dispensary: us.fullscript.com/welcome/drseay***
>
> *Disclosure: Root Health earns a portion of dispensary purchases. Nothing in this book was included, excluded, ranked, or dosed based on what it earns, and you can buy any of it anywhere.*

Recommended Lab Tests

Test	Platform	Priority
Organic Acids Test	Vibrant Wellness	High: D-arabinitol is the gold standard Candida marker; run first
Gut Zoomer 3.0	Vibrant Wellness	High: quantifies Candida species, microbiome context, and gut barrier
GI-MAP	Diagnostic Solutions	High: PCR-based Candida quantification; run alongside OAT for complete picture
Micronutrient Panel	Vibrant Wellness	High: identifies the deficiencies Candida has created and that perpetuate it
Food Zoomer	Vibrant Wellness	Moderate to high: identifies Candida-driven food sensitivities; run in month 1
Wheat Zoomer	Vibrant Wellness	Moderate: quantifies gluten-driven permeability that is synergistic with Candida
Vibrant Mycotoxin Profile	Vibrant Wellness	High: confirms gliotoxin production; rules in or out mold co-exposure
Candida + IBS Profile	Vibrant Wellness	High: serum IgG/IgA Candida antibodies; key when OAT is elevated but stool PCR is negative; $300 blood draw

Ready to Resolve Your Candida for Good?

If you are ready to move beyond temporary relief and address the root causes of your Candida overgrowth, the Root Discovery Session is where we begin.

In 90 minutes we will review your complete health history, explain which tests are the right starting point for your specific situation, and map an individualized protocol based on your root causes.

The cost of the session credits fully toward any program you choose.

Schedule at rootcauseunlocked.com or call our Garner, NC office.

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